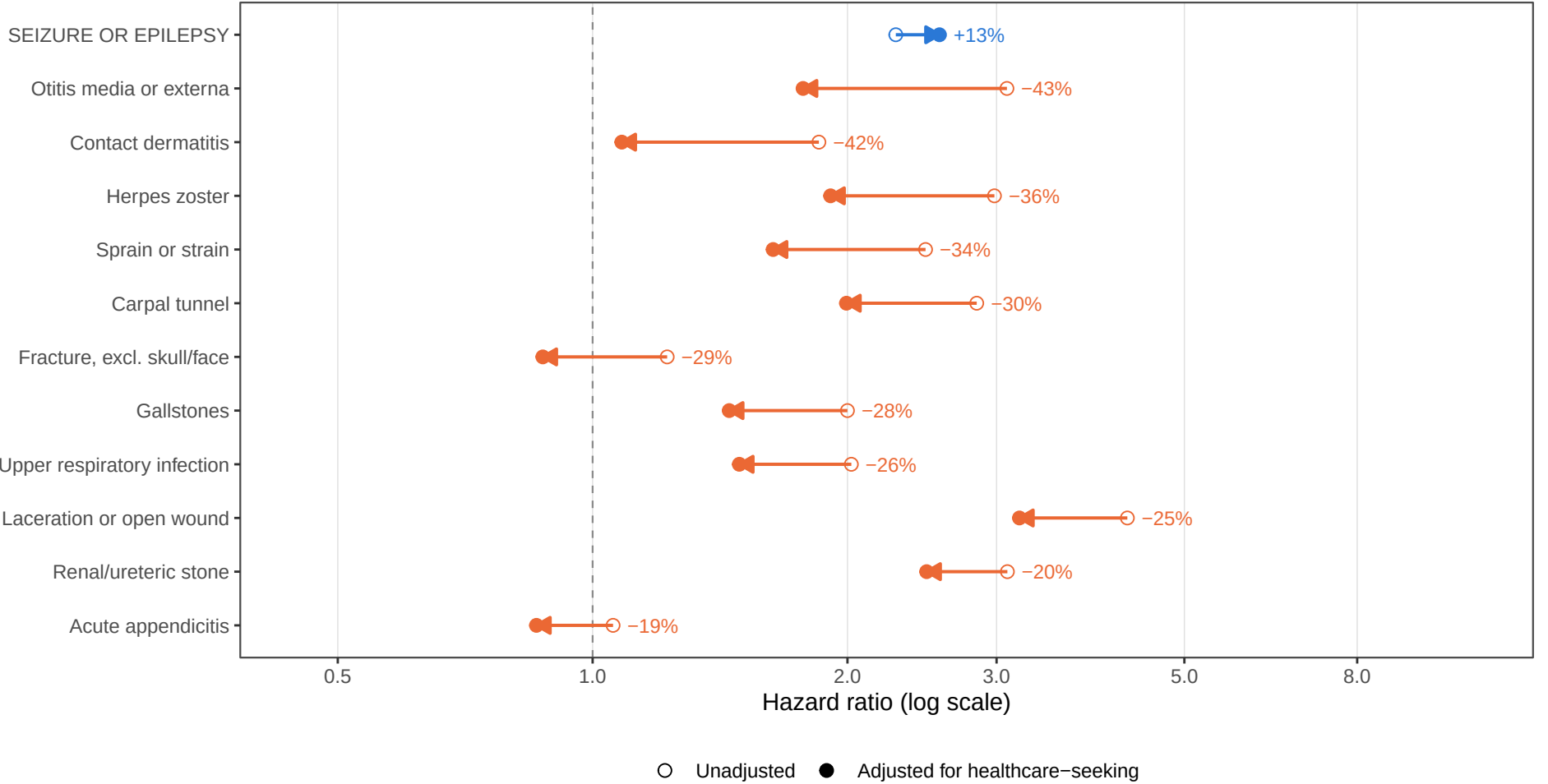


The seizure association is the only one that does not shrink

Arrows run from the unadjusted hazard ratio to the hazard ratio adjusted for pre-index visit count.



Pre-index visit count proxies healthcare-seeking. It is not caused by the outcome, so adjusting for it is legitimate; post-index visits ARE caused by the outcome, and adjusting for those would be collider bias. All eleven negative controls move toward the null, by 19% to 43% (median 29%). The seizure outcome moves the other way, by +13%. It is the only one of twelve to do so. If the seizure excess were produced by patients with IIH simply being seen more often, it should have shrunk like the rest. This does not exclude confounding by healthcare contact altogether: the adjustment is imperfect, and a single pre-index count cannot capture everything about how intensively a patient is observed.